

Choroidal melanoma and rhegmatogenous retinal detachment

Hesham Lakosha,* MD, FRCS(Edin); Rand Simpson,* MD, FRCSC;
David Wong,† MD, FRCSC

Retinal breaks with rhegmatogenous retinal detachment are rare in patients with choroidal melanoma. It has been estimated that less than 1% of eyes with this tumour will have a rhegmatogenous retinal detachment.¹ In a review of the English-language literature, we found 22 such cases.²⁻¹³ The relation of the tumour to the retinal break is obscure.

CASE REPORTS

Case 1

A 74-year-old woman was referred to the Ocular Oncology Service with retinal detachment in association with a choroidal melanoma in her left eye. Six months earlier she had noted photopsia in that eye, followed by progressive deterioration of vision. She reported that the vision in her left eye had been blurry for a long time and the cause was unclear. She had controlled hypertension.

Ocular examination showed a visual acuity of 20/50 with +0.25 in the right eye and counting fingers with +0.25 in the left. The intraocular pressure, as measured by applanation tonometry, was 18 mm Hg bilaterally. There was a relative afferent pupillary defect in the left eye. Funduscopy of the left eye with the pupil dilated revealed a pigmented mass at the posterior pole measuring 10.5 mm by 6.5 mm in basal diameters. Lipofuscin pigment was visible on the surface of the lesion. An infe-

rior hemispheric retinal detachment was noted involving the retina overlying the tumour. The detached retina had an undulated surface (Fig. 1). There was a small equatorial retinal hole on one side of a sclerosed segment of a blood vessel, about 2 mm from the inferior margin of the tumour. The vitreous was clear. The fellow eye displayed long-standing branch retinal vein occlusion that appeared nonperfused on fluorescein angiography.

Standardized echography showed a dome-shaped tumour with a thickness of 3.2 mm. Fluorescein angiography revealed dilated retinal blood vessels with staining of their walls and areas of retinal capillary nonperfusion inferior to the tumour (Fig. 2). A diagnosis was made of a medium-sized choroidal melanoma with an ischemic retinal hole. Metastatic investigation did not show any abnormalities, and the patient underwent iodine-125 brachytherapy.

Four months postoperatively the woman's visual acuity was stable, at counting fingers. Fundus examination remained unchanged, although ultrasonography showed a decrease in the tumour height to 2.9 mm.

Case 2

A 50-year-old man presented to his ophthalmologist

From *the Ocular Oncology Service, Princess Margaret Hospital, Toronto, Ont., and †the Vitreoretinal Service, Saint Michael's Hospital, Toronto, Ont.

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Correspondence to: Dr. Rand Simpson, Ocular Oncology Service, Princess Margaret Hospital, 610 University Ave., Toronto ON M5G 2M9; fax: (416) 946-2189

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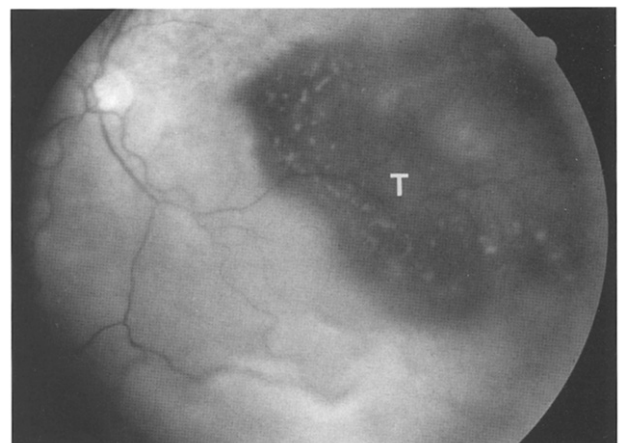


Fig. 1—Case 1: Fundus photograph, showing choroidal melanoma (T) and inferior retinal detachment. Note undulated appearance of detached retina.